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Proposal Title:	An interdisciplinary approach to coping with Parkinson's disease and its relationship with well-being: psychological, neurological, and social variables

Chile is a country ageing at a significant rate. People 60 years old and up represent 19.7% of the population, amounting to 3,8 million people (CAsEN, 2020). The growth of this population will result in major challenges, making it necessary to determine how to foster their quality of life and respond to their well-being expectations. It is well-known that ageing processes entail changes that cause people to deal with undesired situations, such as organic decline and increased risk of losing good health. Becoming sick and ultimately being dependent is one of the losses that older adults (**OAs**) most commonly highlight (Thumala, 2014), and for which there tends to be little preparation (Arnold, et al., 2021). Neurodegenerative diseases stand out among the most disabling in old age. They are chronic, progressive, trigger severe limitations and, to date, there is no cure for them. Among these, one of the most prevalent is Parkinson's disease (**PD**) and mostly affects OAs (MINSAL, 2010; Kalia & Lang, 2015). There are 6,3 million people living with PD worldwide, and Chile is the Latin American country where prevalence has increased the most (Leiva et al., 2019), reaching 40 thousand patients (Said, 2018). Those who suffer from PD are aware of the progression of their disease and have the permanent **challenge of coping with the loss of health**. PD not only affects the patient: those around him are also exposed to various psycho-emotional, economic, and family disorders that interact with each other (MINSAL, 2010). Being an expensive and limiting pathology, PD affects public health, especially given the increasing volume of OAs.

Studies on how people cope with PD are still scarce and inconclusive, cover a wide range of actions, some of which are highly specific, refer to various outcomes, and do not offer clear classification schemes. This makes it difficult to compare results and accumulate knowledge in this field. Furthermore, we consider that the inconclusive results so far may be due to the absence of an interdisciplinary approach capable of addressing the complexity of PD coping strategies, which should be regarded as a phenomenon configured by the psychological and neurological/neurocognitive characteristics of PD patients (**PDPs**) and the social factors influencing them, all of which can have an impact on the variability of the results reported in the literature.

Our work is based on the coping model advanced by Brandtstädter (2009), which comprises two general coping modes –assimilation and accommodation– that make it possible to classify a range of specific responses. In addition, we selected psychological (personality traits, self-efficacy), social (socio-demographic factors and social integration), and neurological/neurocognitive variables (stage of PD, presence of cognitive impairment, and/or apathy). To examine well-being, we will measure levels of subjective well-being, psychological well-being, and depressive symptomatology. Our **OBJECTIVE** is to evaluate the relationship between coping strategies (levels of assimilation and accommodation) and well-being (subjective, psychological, and presence/absence of depressive symptoms) in a population of PDPs, and to examine their interactions with a set of psychological, social, and neurological/neurocognitive variables. **METHOD:** We expect PDPs to participate as relevant actors in the knowledge production process, given their status as experts in living with PD. In the *first stage*, we will conduct a correlational, cross-sectional study of a quantitative nature, using as our information source the responses of at least 200 PDPs (estimated sample) to multiple instruments to be administered. Towards the end of this phase, we will conduct a complementary screening process to characterize, in a qualitative manner, the results yielded by the quantitative analysis that best illustrate these results. This screening process will focus on identifying participants who represent coping strategies associated with the highest and lowest levels of well-being, according to a set of psychological, social, and neurological/neurocognitive variables. In the *second stage*, we will invite the participating PDPs to attend a presentation and discussion of the results in which –through the integration of their observations into our analysis– we will generate materials and establish guidelines for designing interventions; for instance, a pilot workshop on how to cope with PD to be implemented and evaluated in a future study. **EXPECTED RESULTS:** Characterization of copings best related to well-being and for which types of PDPs they are, according to their psychological, neurological/neurocognitive, and social characteristics. The **relevance** of our proposal lies in its goal of generating knowledge that will inform public health policies, specifically through the design of mental health prevention and treatment programs –e.g., clinical, psycho-educational, groups, or community initiatives– benefiting a growing population of PDPs and their caregivers, taking into account the diversity of those who live with this disease. Lastly, the **feasibility** of this study is grounded in the experience of all three researchers as members of interdisciplinary projects (they belong to the Interdisciplinary Psychogerontology Nucleus, U. de Chile, led by the principal researcher), the support of health centers where PDPs are diagnosed and monitored –making it easier for us to obtain the sample–, and access to the necessary technical and support personnel.